

WEIGHT LOSS

Monitoring, Labs & Follow-Up Schedule

The cadence, the panels, and what happens when a patient stops attending

DOCUMENT WL-009	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians and clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Follow-up is what makes this a medical program rather than a supply arrangement. This protocol sets the visit cadence, the laboratory panels and their intervals, the escalation pathway for abnormal results, and the response when a patient with an active prescription disengages.

PRESCRIPTIONS DO NOT OUTLIVE MONITORING

A patient who cannot or will not attend monitoring does not continue on therapy. The prescription is contingent on the follow-up, and staff should be able to say so without needing to escalate the conversation.

2 Visit Cadence

Phase	Interval	Required at each visit
Baseline	Before first dose	Full evaluation per WL-003, measured vitals and anthropometrics, baseline labs, consent, education.
Titration phase	At or before each dose escalation	Weight, BP, resting heart rate, GI symptom grading, adherence, intake and protein, escalation decision recorded per WL-004.
Early maintenance	At the interval set by the medical director	As above, plus body composition or circumference, and interim labs where indicated.
Established maintenance	At a defined longer interval, not exceeding the maximum set by the medical director	As above, plus full review of goal, therapy duration and exit planning per WL-010.
Annual	Every 12 months on therapy	Comprehensive review: full labs, contraindication re-screen, disordered eating re-screen, consent refresh, and documented decision to continue.

TELEHEALTH VISITS

Where follow-up occurs by telehealth, the practice must still obtain objective measurements — a documented home weight, and blood pressure where the patient has a cuff — and must define which assessments require an in-person visit. Record the modality of every encounter. State telehealth rules for prescribing vary and must be confirmed.

3 Laboratory Monitoring

The panel below is a framework. Your medical director sets the practice panel, the intervals, and the thresholds, taking into account each patient's comorbidities.

FRAMEWORK — MEDICAL DIRECTOR TO FINALIZE

Test	Baseline	Repeat	Rationale
Comprehensive metabolic panel	Yes	At set intervals	Renal and hepatic function; electrolytes, particularly where GI losses occur.
HbA1c	Yes	At set intervals	Glycemic status; identifies undiagnosed diabetes and tracks improvement.
Fasting glucose	Yes	As indicated	Baseline glycemic assessment.
Lipid panel	Yes	At set intervals	Cardiovascular risk tracking.
Complete blood count	Yes	As indicated	General baseline.
TSH	Yes	As indicated	Thyroid dysfunction as a contributor to weight change.
Lipase	As indicated	If symptoms arise	Not routinely required; obtained where pancreatitis is suspected.
Pregnancy test	Where applicable	Per medical director	Before initiation in patients who could become pregnant.
Vitamin and micronutrient studies	As indicated	As indicated	Where intake is very low or deficiency is suspected.

4 Abnormal Results & Escalation

- 1 All results reviewed by a clinician** within a defined timeframe of receipt. Results are not filed unreviewed, and 'no news' is not a system.
- 2 Classify** the result: within expected range, abnormal requiring monitoring, abnormal requiring action, or critical.
- 3 Critical results are actioned the same day**, with the patient contacted directly rather than by message.
- 4 Notify the patient** of every result, normal or otherwise, within the practice's stated timeframe.
- 5 Communicate with the primary care physician** for any finding outside the scope of this program — with the patient's consent, and in writing.
- 6 Hold or stop therapy** where the result warrants it, per WL-004.
- 7 Document** the review, the classification, the action, the notification and the onward referral.

THE UNREVIEWED RESULT

An abnormal result sitting unread in an inbox is one of the most common and most indefensible failures in outpatient medicine. Assign a named owner for result review, with named cover for absence, and audit the queue on a schedule.

5 Non-Attendance & Lost to Follow-Up

A patient on an active prescription who stops attending is a clinical risk and an accountability problem. Define the response rather than letting the chart go quiet.

- ☐ Missed appointments logged, with a contact attempt made and recorded.
- ☐ A defined number of contact attempts through more than one channel before a patient is considered disengaged.
- ☐ No further prescription issued or product dispensed once the follow-up interval has lapsed beyond the defined limit.
- ☐ Written notice to the patient explaining that therapy cannot continue without monitoring, and what they must do to resume.
- ☐ Primary care physician notified where clinically appropriate and consented.
- ☐ Formal discharge from the program after a defined period, with a discharge letter and summary per WL-010.
- ☐ Chart flagged so a lapsed patient is re-evaluated rather than simply re-supplied.

6 Quick Reference

QUICK REFERENCE CARD**Monitoring essentials**

1. **Baseline labs before the first dose.** No exceptions.
2. **Weight, BP and resting heart rate** at every visit — measured.
3. **GI grading, adherence, protein** every visit.
4. **Named owner for result review**, with cover, and an audited queue.
5. **Critical result → same-day direct contact.**
6. **Findings outside program scope → write to primary care.**
7. **No monitoring, no prescription.** Say it plainly and early.
8. **Annual comprehensive review** with a documented decision to continue.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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